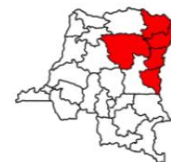




TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°093/MVEBDB/15/08/2026

Reporting date: August 15, 2026

Publication date: August 16, 2026



HIGHLIGHTS (last 24 hours)

Over the past 24 hours, 101 new confirmed cases of Ebola Virus Disease (EVD), including 33 community deaths, have been recorded in the provinces of Ituri (89 cases), North Kivu (8 cases), Haut-Uélé (3 cases), and Tshopo (1 case). No new health zones have been affected.

ACCUMULATIVE CAS 4 945	ACCUMULATIVE CONFIRMED DEATHS 2 325	 LETHALITY 47,0%	PATIENTS IN ISOLATION / CTE 730	ACCUMULATIVE HEALED 1 040	TRACKING RATES CONTACTS (Of the Day) 85,3 %
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uele, Ituri, North Kivu,
South Kivu, Tshopo & Lower Uele



55 Health Zones

Affected Out of 151

(36,4%)



Summary of key points from the last 24 hours

Over the past 24 hours, 101 new confirmed cases and 33 community deaths from Ebola Virus Disease (EVD) have been reported in the provinces of **Ituri (89 cases)**, **North Kivu (8 cases)**, **Haut Uélé (3)**, and **Tshopo**.

(1) Ituri accounts for approximately two-thirds of the new cases.

At the end of the day on August 15, 2026, 34 patients were declared recovered after receiving two negative Ebola virus (EV) test results (34 in Ituri). However, 20 confirmed deaths within Ebola treatment centers were recorded, including 17 in Ituri and 3 in North Kivu, bringing the total number of deaths in the last 24 hours to 53.

From the start of the epidemic until August 15, 2026, the Democratic Republic of Congo recorded 4,945 confirmed cases, including 2,325 deaths, representing a case fatality rate of 47.0%. Ituri remains the epicenter of the epidemic, accounting for 84.8% of cumulative confirmed cases and 88.1% of new cases reported in the last 24 hours.

Overall, 55 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (1/11).

Distribution of confirmed cases and deaths by affected province

(As of August 15, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4194	1 838	43,8%	28/36 (77,8 %)	89
North Kivu	596	417	70,0%	12/34 (35,3 %)	8
Haut-Uélé	138	62	44,9%	6/13 (46,1 %)	3
Boss	13	6	46,2%	7/23 (30,4 %)	1
South Kivu	3	1	33,3%	1/34 (2,9 %)	0
Bas Uélé	1	1	100,0%	1/11(9,1%)	0
Total	4 945	2 325	47,0%	55/151 (36,4 %)	101

"A case from Makiso (Tshopo), recorded on August 14, had been omitted from the SitRep 092 cumulative total."

Confirmed cases and deaths by province and health zone (situation as of August 15, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4194	1838	43,8%	89	30	17	47
Give	11	1	9,1%				0
Underwood	7	6	85,7%				0
Ariwara	7	2	28,6%				0
Aungba	11	5	45,5%				0
Bamboo	109	25	22,9%				0
have	1	1	100,0%				0
Bunia	1157	352	30,4%	41	14		14
Ladies	23	9	39,1%				0
Drodoro	13	6	46,2%				0
Fireworks	64	30	46,9%	1	0		0
Gethy	4	1	25,0%				0
Flounder	2	0	0,0%				0
Kilo	31	12	38,7%				0
Team	48	32	66,7%	3	1		1
Liter	173	98	56,6%	1	1		1
Logo	11	5	45,5%				0
Fight	15	4	26,7%	1	0		0
distributed	4	1	25,0%				0
Washing	14	7	50,0%	1	1		1
Darkness	23	14	60,9%	1	1		1
Mangala	156	88	56,4%	15	5		5
Mongbwalu	585	282	48,2%	1	0		0

Nanny	164	96	58,5%	3	0		0
Nizi	534	239	44,8%	10	3		3
Legumes	120	35	29,2%				0
Jungle	9	4	44,4%				0
Rwampara	849	328	38,6%	11	4		4
Tchomia	49	25	51,0%				0
A valves	THAT	130	THAT	THAT	THAT	17	17
North Kivu	596	417	70,0%	8	3	3	6
Me	91	68	74,7%	1	0	2	2
Butembo	111	92	82,9%	1	1		1
Ten	1	0	0,0%				0
Kalunguta	12	5	41,7%				0
Cut	285	193	67,7%	5	2	1	3
Kyondo	15	12	80,0%				0
Lubero	1	1	100,0%				0
Be careful.	2	1	50,0%				0
Masereka	7	4	57,1%				0
The museums	62	35	56,5%	1	0		0
Oicha	5	4	80,0%				0
Vuh	4	2	50,0%				0
Haut-Uélé	138	62	44,9%	3	0	0	0
Kill Mangbetu	22	8	36,4%				0
Gombari	1	1	100,0%				0
Mathematics	42	19	45,2%	3	0		0
Lining	19	14	73,7%				0
Beans	1	0	0,0%				0
Ordinary	53	20	37,7%				0
Boss	13	6	46,2%	1	0	0	0
Bafwasende	1	0	0,0%				0
Kabondo	3	2	66,7%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	4	3	75,0%	1	0		0
Mangobo	2	1	50,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	0	0,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	1	1	100,0%	0	0	0	0
Blind	1	1	100,0%				0
Total	4945	2325	47,0%	101	33	20	53

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

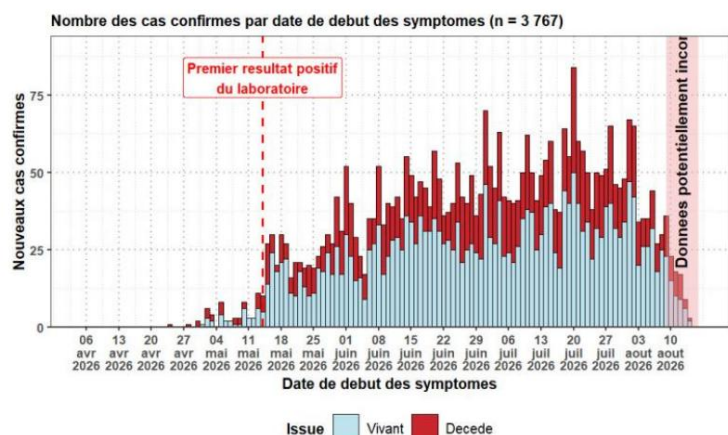
NA : Not Applicable

Status of alerts notified by province, as of August 15, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	4	3	7	4	2	0	1	6	3
Ituri	693	72	765	179	62	375	10	169	165
North Kivu	906	38	944	95	38	803	0	131	95
South Kivu	14	0	14	0	0	6	0	0	0
Tshopo	ND	ND	ND	ND	ND	ND	ND	ND	ND
Total General	1617	113	1 730	278	102	1184	11	306	263

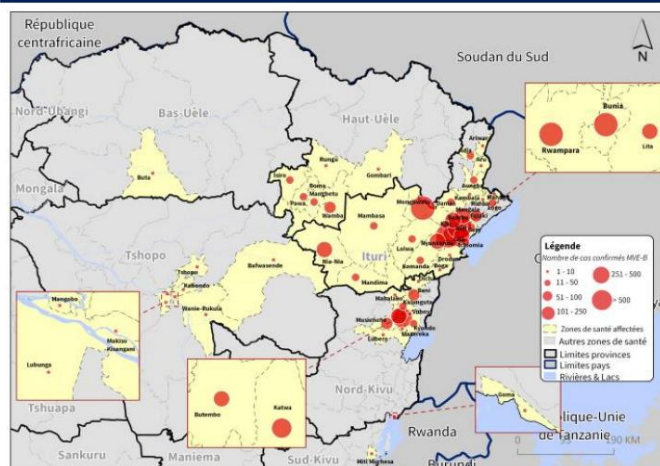
*Tshopo alert data is not available for the period considered.

Evolution of cases by date of symptom onset over the last 21 days



Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de début de symptômes renseignés, n = 3 767.

MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1.Coordination

1.1.1. Main Actions

- **North Kivu** : Official handover of an ambulance to the Musienene Health Zone (Government support), participation in the monitoring and coordination meeting of the Beni Health Zone and working sessions with SONAHU, CARITAS/Goma, FEPSI and WHH on their accreditations and complementary activities.
- **Ituri/Tshopo**: Joint visit by the Incident Manager, the General of the 32nd Military Region, the Haut-Uélé delegation, and the PoE pillar to Nia-Nia to engage with the community and reinforce security at the checkpoint located on the border between Ituri and Haut-Uélé; launch of the systemic evaluation of the PoE/PoC pillar in five provinces (Ituri, North Kivu, Haut-Uélé, Bas-Uélé, and Tshopo).

Epidemiological surveillance

1.1.2. Main Actions

By the end of the day on August 15, 2026, 1,730 alerts had been recorded, of which 1,575 (91.0%) were verified in the four reporting provinces. Of these, 380 (24.1%) were confirmed as suspected cases and 306 (80.5%) were investigated, with 263 (85.9%) being referred to COVID-19 treatment centers. Tshopo province remained non-reporting.

The proportion of contacts monitored in the last 24 hours has risen to 85.3% (20,791 seen out of 24,383 to be monitored), above the 85% threshold: Ituri 89.8% (13,582/15,126), Tshopo 100% (439/439), and North Kivu 79.0% (6,056/7,662). Haut-Uélé 61.8% (714/1,156) did not submit consolidated data (4 health zones did not report), and South Kivu no longer has any active contacts. Thirty-three (33) community deaths confirmed by SWAB were recorded (30 in Ituri and 3 in North Kivu).

Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Nine (9) alerts were detected at the various PoE/PoC points in the last 24 hours: five in Ituri and four in North Kivu. Details of the actions taken at each checkpoint are presented below, by province.

Monitoring of indicators at PoE/PoC by province, as of August 15, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Global
Proportion of activated PoCs and Enhanced PoE	100 % (60/60)	100 % (18/18)	100 % (9/9)	35,0 % (7/20)	87,9 % (94/107)
Number of people who have switched to PoE/PoC	132 493	78 435	84 432	3 481	298 850
% of travelers screened % of	96,0 % 97,21 %	100 %		99,3 % 97,5 %	
travelers who washed their hands	96,0 % 97,21 %	100 %		99,1 % 97,5 %	
% of travelers made aware	99,4 % 97,21 %	100 %		100 %	99,0 %
Number of alerts notified	5	4	0	0	9
Number of validated live alerts	1	3	0	0	4
Number of bodies recovered today	4	0	0	0	4
Number of contacts to Problems intercepted today	0	0	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100 % (1/1)	100 % (3/3)	0	0	100 % (4/4)
% of the lifeless bodies swabed	75 % (3/4)	0	0	0	75 % (3/4)

Indicators	Ituri	North Kivu	South Kivu	Boss	Global
Number of positive results of referred suspected cases	0	0	0	0	0

1.2.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Tshopo (35% activated, 57.1% completeness), due to a lack of training, supervision and logistical and financial support; in Ituri, only one strategic site out of 11 is working without interruption.
- Constraints affecting the functionality of PoE/PoC: non-payment of service providers in Ituri, North Kivu and Haut-Uélé, which led to a strike that deprived six PoCs in Haut-Uélé of reporting; 2,187 refusals of screening in North Kivu (2.78%) and low screening at the Pont-Shari PoC in Ituri due to lack of involvement of uniformed men; insufficient thermal flash (11/19) and means of transport for supervision in Tshopo.

1.3. Laboratory

1.3.1. Main actions

Ituri recorded 89 new positive results (59 living cases and 30 deaths) out of 288 new samples received and analyzed (positivity rate: 30.9%). Additionally, 8 samples came back positive out of 79 analyzed. Tshopo recorded one positive result, while North Kivu confirmed 8 new cases, including 3 deaths, out of 133 analyzed samples (6.0%), Haut-Uélé confirmed 3 cases (living cases) out of 9 samples (positivity rate: 33.3%), and Tshopo confirmed 1 case out of 6 samples received and analyzed (16.6%).

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 171 rings were expected (54 postponed from the previous day and 117 confirmed cases retained), and 21 rings were actually opened; 77 sites were identified and 64 decontaminated, with 13 gaps exceeding 48 hours; 83 households were identified and 70 decontaminated, and 9 IPC kits were distributed. The documented causes were insufficient decontamination teams (Bambu-mine, Mangala), insufficient supplies (Mandima), resistance from families (Kilo), unrecorded addresses, and a strike by decontamination workers.
- **In North Kivu**, the briefing for the EDS teams in the Beni health zone has concluded: 39 death alerts were received, with 12 postponements; 37 bodies were recovered by swab, and 37 EDS were carried out; two failures were reported, linked to community resistance in the Beni and Kalunguta health zones. The death alerts originated primarily from Beni (20; 51.3%), Katwa (7; 17.9%), and Butembo (6; 15.4%).
- **In Haut-Uélé**, 8 death alerts were recorded in the health zones of Wamba, Pawa, and Isiro (3 reported and 5 alerts issued daily) out of 6 complete blood counts (76.7%) and 1 body collected by swab; one failure was reported—a body taken by the community in the Tuluba health area (Isiro health zone) remains unswabbed. Seven rings were opened, with decontamination of healthcare facilities in Pawa (General Referral Hospital), Wamba (Mama Kayenga Clinic and Treatment Center), and Isiro (Mayogo Hospital, Tuluba Health Center, and Tely Health Center).

1.4.2. Challenges

- Weak implementation of IPC standards: limited ring opening in Ituri (21 out of 171) and incomplete decontamination (70 out of 171 sites) due to strike by decontamination workers, insecurity, refusals and addresses not found; completeness of IPC reporting limited to 9 out of 14 health zones in Ituri; very low IPC scores in Tshopo (CS Papa Freddy 12.6%, Clinique William 11.6%, Pavillon militaire 12.9%); hygiene kits not available in all health zones of Haut-Uélé; removal of a body by the community in the Tuluba health area (Isiro health zone).

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, 30 out of 32 ambulances are available (94%), with 17 in use (57%) and 28 out of 28 successful missions (100%). Emergency treatment centers (CTE/CT) recorded 116 new admissions. All suspected cases, including 1 PPL (doctor from Lita General Referral Hospital) and 7 in advanced stages — and 116 discharges, including 34 recoveries, 51 non-cases, 8 escapes, and 23 deaths (11 of which were confirmed). Occupancy reached 548 patients out of 991 beds (55.3%), with 236 confirmed cases out of 497 beds (47.5%) and 312 suspected cases out of 494 beds (63.2%); 31 patients are in critical condition, and the cumulative number of recoveries is 955. The Ebola Treatment Centers (ETCs) in Bambu (confirmed and suspected cases), Fataki and Nizi (confirmed cases), as well as Aru, Kilo, Logo, and Kigonze (suspected cases), are at capacity. 504 patients received a hot meal (99%).

In North **Kivu**, 32 new admissions were recorded (1 confirmed at the Katwa Ebola Treatment Center and 31 suspected cases), with 42 discharges, including 36 non-cases, 1 escapee in Masereka (total number of escapees: 22), and 5 deaths, including 3 confirmed cases (1 in Katwa and 2 in Beni). Currently, 170 patients are hospitalized (28 confirmed and 142 suspected cases) in 206 beds, representing an occupancy rate of 82.5% (43.8% for confirmed cases and 100% for suspected cases). Five ambulances are available, and 556 patients received a hot meal (100%).

- **In Tshopo**, a new admission was recorded (29-year-old woman, Kabondo health zone) and 8 patients are in isolation at the Kisangani CTE HC (3 confirmed and 5 suspected) for 12 beds, i.e. 66.7% occupancy (75% for confirmed and 62.5% for suspected); 20 high-risk contacts are being managed, including 11 under experimental treatment, and 24 hot meals were served with the support of MSF.

- **In South Kivu**, 7 suspected cases are being monitored in isolation at the Katana Health Zone (FOMULAC); no confirmed cases are being admitted to the CTE and the province has gone 66 days without a new confirmed case since June 10, 2026.

1.5.2. Challenges

Insufficient availability of referral resources (2 ambulances out of service in Ituri: call center and Bambu) only 5 ambulances in North Kivu and need for 10 more ambulances in Haut-Uélé) and low capacity of some CTE/CT: the CTEs in Bambu (confirmed and suspected), Fataki and Nizi are saturated, the occupancy of suspected beds reaches 100% in North Kivu and the suspected cases in Butembo, Masereka, Vuhovi and Kalunguta are isolated in health facilities that are not transit centers; absence of standardized CTE in the six affected health zones of Haut-Uélé and reduced capacity of the CTE HC in Kisangani (12 beds).

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 810 new contacts were identified around the day's cases; 4,121 people were reached during home visits by 270 community health workers (Nia-Nia and Mangala health zones), 2,481 during community dialogues (Nia-Nia and Nyankunde), and 4,623 during

Mass awareness campaigns were conducted in Nyankunde and Nia-Nia, involving 25 people engaged in advocacy, resulting in 22 radio programs produced, 6 public service announcements broadcast, 36 community radio stations involved, and 42 IEC (Information, Education, and Communication) materials distributed. CREC supported the listing of 14 contacts in Nia-Nia and 10 contacts in Rwampara, the raising of awareness among 4 affected families in Rwampara regarding the EDS (Emergency Health Data) program, and the raising of awareness among 89 affected family members in Mongbwalu, Nizi, Mangala, and Bambu. Eight out of 28 health zones in Ituri and two in North Kivu reported community concerns in the last 48 hours, representing 19% of the health zones reached.

- **In North Kivu**, mass communication on Ebola prevention was conducted among soldiers gathered at the Mambango parade, their wives and widows of soldiers; communication around 20 confirmed cases reached 113 household heads, 17 rumors were clarified (2 gaps) and 14 community incidents were recorded, all unresolved and located in the Musienene health zone.
- **In Haut-Uélé**, 289 people were reached during home visits to 66 households, 289 during educational talks (Wamba and Isiro health zones), and 218 during mass awareness campaigns; 5 radio stations broadcast prevention messages and 41 IEC (Information, Education, and Communication) tools were distributed (total of 340), with CREC (Community Resource and Coordination Center) coverage in 4 out of 5 health zones (80%). A joint Surveillance-CREC field visit was conducted following a community alert in the Digi health district (Pawa health zone). In Tshopo, a working session was held with the Provincial Director of Primary, Secondary, and Vocational Education (EPSP) on Ebola prevention and response in schools, and a reported resistance to the military building was lifted.

1.1.1. Challenges

- Persistent community challenges: refusal of EDS in the health districts of N'sele and Tuluba in Haut-Uélé, with removal of a body by the community in Tuluba; 14 unresolved community incidents in North Kivu, all in the Musienene health district, and persistent rumors in the Katwa health district (alleged distribution of "infected" Ebola tablets); in Ituri, repeated resistance to EDS in Mungamba and low reporting of community concerns (19% of health districts affected); awareness posters not translated into local languages in Haut-Uélé; low coverage of SMSPS interventions (4 out of 14 health districts having notified cases in Ituri).

1.7. Logistics

1.7.1. Main actions

In Ituri, training for CTE Sota service providers continues, and the needs include four glove boxes (for the health zones of Fataki, Bambu-mine, Lita, and Rwampara) as well as cryotubes and cryoboxes. Four mobile laboratories are planned for these same health zones.

In North Kivu : continued construction of the triage unit at the Vuhovi General Referral Hospital (ULB Cooperation) and rehabilitation work on the CT at the Musienene General Referral Hospital (MSF France), development of the workspace for the PCI and EDS teams in Musienene, repair of the V101 mobile unit at the Mutwanga garage and inventory of medicines and supplies at the warehouse.

In Haut-Uélé : reception of a flight carrying 2,650 kg of nutritional support and IPC (Infection Prevention and Control) supplies and equipment (IOM and WFP), 600 L of fuel (MEMISA donation) for the health zones of Pawa and Boma-Mangbetu, as well as an ambulance and a pickup truck donated by the provincial government to the health zones of Rungu and Wamba. In Tshopo: visit to the isolation site under construction at the Kabondo General Referral Hospital and the Madula isolation unit, monitoring of the construction work at the standardized treatment center in Simisimi, and support for the transport of IPC supplies.

1.7.2. Challenges

1.8. Persistent logistical needs to support the expansion of the response: completion of the extensions of the CTEs in Nizi, ISTM and Bambu in Ituri; stock shortages reported in Tshopo (zero head coverings and surgical masks, 349 kg of chlorine available for a need of 1,800 kg, hydroalcoholic gels and protective glasses almost exhausted) and alert on the fuel stock in Haut-Uélé; unmet mobility needs (178 motorcycles and 7 vehicles missing in Tshopo, permanent vehicle for security teams and EDS in North Kivu and Haut-Uélé); cascading storage of inputs in Haut-Uélé.

1.9. Security

1.9.1. Main actions

- Securing the response activities continued in North Kivu with two actions to maintain public discipline carried out 100%: accompanying and providing security coverage for the EDS team for a community death (swab) in the Ruenda district, Bulengeru commune, to the morgue of the Katwa General Referral Hospital, and accompanying the EDS team for the burial of a confirmed case in Vulimira (Ndwani and Maisafi, Oicha Health Zone); A briefing was also held for agents involved in the response on compliance with instructions and punctuality, and the provisional mapping of accommodation sites and recommended neighborhoods in the cities of Beni and Butembo was updated for August 2026. In Ituri, security at the checkpoint located on the border between Ituri and Haut-Uélé (Nia-Nia) was reinforced with the support of the 32nd military region.
- Persistent insecurity and community resistance during dignified and secure burials, limiting access for response teams: EDS not carried out at the PoC of Epulu/Zunguluka (ZH of Mambasa, Ituri) for security reasons, insecurity in the villages opening rings in Ituri, attack on the nurse in charge of the AS Apodo (ZH of Gombari, Haut-Uélé), growing insecurity in Baego (ZH of Bafwasende, Tshopo) and seventeen health areas identified as problem areas in North Kivu; non-take-up of police elements assigned to the CTE of Katwa and Kitatumba.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) continued in the affected provinces. No daily data was submitted by the pillar for the period under review; activities reported in the previous cycle are continuing.

1.1.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse remains insufficient across all areas covered by the response.

MESSAGES CLES

- Ebola virus transmission remains active, with 101 new cases including 33 deaths community and 20 intra-CTE recorded in the last 24 hours, bringing the national total to 4,945 confirmed cases and 2,325 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 84.8% of cumulative confirmed cases and 88.1% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows 80.5% of suspected cases investigated while the follow-up rate (85.3%) reaches the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



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